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GUIDELINE ON RESCHEDULING OF A SUBSTANCE OR MEDICINE

This document is intended to provide guidance to applicants on the scheduling of substances submitted for registration as medicines. It represents the South African Health Products Regulatory Authority's current thinking on the safety, quality and efficacy of medicines and takes into account the country's obligations in terms of international agreements. It is not intended as an exclusive approach. The Authority reserves the right to request any additional information to establish the scheduling status of a medicine in keeping with the knowledge current at the time of evaluation. Alternative approaches may be used but these should be scientifically and technically justified. It is important that applicants adhere to the administrative requirements to avoid delays in the processing and evaluation of applications.

Document History

Final Version	Reason for Amendment	Effective Date
1	First issue	30 May 2022
2	• Alignment to the latest SAHPRA guideline template • Introduction to the Rescheduling Application Form and general process alignments.	March 2026

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Glossary

Abbreviation/ Term	Meaning
API	Active Pharmaceutical Ingredient
SAHPRA	South Arican Health Products Regulatory Authority

APPROVED

1. INTRODUCTION

This guideline is intended to provide applicants, Holders of Certificates of Registration (HCR) with guidance on the criteria applied by the South African Health Products Regulatory Authority (“SAHPRA” or “the Authority”) when applying for the rescheduling of a medicine or substance.

1.1 Purpose

This purpose of this document is to provide guidance to applicants on the rescheduling of registered medicinal products.

1.2 Scope

This Guideline contains guidance on the suitability of rescheduling medicines or substances. This guideline should be read in conjunction with the Medicines and Related Substances Act, 1965 (Act 101 of 1965) (herein referred to as the Medicines Act), as amended, and the Regulations issued in terms of this Act; and the latest version of the Consolidated Schedules.

2. LEGAL PROVISION

In terms of the Medicines Act, a ‘Scheduled substance’ is defined as follows: “means any medicine or other substance prescribed by the Minister under section 22A”. Section 22A(2) empowers the Minister of Health “on the recommendation of SAHPRA”, to make the Schedules referred to in that section, Section 37A also empowers the Minister, again on the recommendation of SAHPRA, to amend the Schedules by notice in the Government Gazette. The official Schedules shall therefore always be those that have been published in the Government Gazette or amended by subsequent notice in the Gazette. All medicines are subject to a scheduling process on the basis of the substances (active pharmaceutical ingredients) they contain.

The primary consideration in scheduling a substance is its safety profile, in relation to the therapeutic indications for its use. Substances may be listed in one or more of eight Schedules.

However, substances may also be listed in more than one Schedule, based on dosage form, route of administration, strength, indication, dose, duration of treatment or a combination of these factors. Where a substance or medicine has been listed in more than one Schedule, the other Schedule(s) in which it is listed is/are indicated in parentheses after the inscription in a particular Schedule. Where exceptions are included in an entry, these have been emphasised by printing the word —**except** in bold type.

3. GENERAL INFORMATION

- 3.1 The application form for Rescheduling of a Substance or Medicine (**GLF-CEM-NS-01A**) is to be used for applications regarding rescheduling of a substance or medicine in line with the Guideline to the Scheduling of Substances and Medicines (**SAHPGL-CEM-NS-02**). The application form can be accessible on SAHPRA website.
- 3.2 An application can be submitted by an applicant who wishes to change the schedule status of a registered medicine.
- 3.3 An independent person/s or association may not apply for a rescheduling of a product. However, the independent persons or association may bring forth data/ information to the regulator to highlight any safety/ efficacy concerns observed by the persons or association for the regulator to assess if a feasible action is required.

NOTE: Safety and efficacy data concerns may be submitted to the regulator independently, where applicable.

- 3.4 The same principles as outlined in the Guideline to the Scheduling of Substances and Medicines (SAHPGL-CEM-NS-02) would be applied in considering an application for rescheduling. Therefore, to justify rescheduling from Schedule 3 or 4 to Schedule 2 or 1, evidence would need to be provided to justify use without a prescription.

If any of the following criteria are met, prescription status would need to be maintained:

- The substance is likely to present a direct or indirect danger to human health, even when used correctly, if used without supervision by a medical practitioner or dentist;
 - The substance is frequently and to a very wide extent used incorrectly or inappropriately, and as a result is likely to present a direct or indirect danger to human health;
 - The safety and efficacy of the substance has not yet been adequately described and requires further investigation.
- 3.5 Likewise, in order to justify a Schedule 0 status, evidence would need to be provided to show that the hazard to health, the risk of misuse and the need for special precautions in handling are minimal, and that wider sale would be a convenience to the consumer.

- 3.6 A product is normally assigned the highest scheduling status appropriate to any one of the registered indications of the product.
- 3.7 If an application is made to reschedule only selected indications of a specific product to a lower Schedule a unique name and a separate registration dossier, where applicable, will be required for such a product.
- 3.8 It must be noted that dual scheduling on packaging, Professional Information and the Patient Information Leaflet is not permitted.
- Only products with indications, strengths, routes of administration and/ or pack sizes applicable to the lower schedule will qualify for the lower scheduling status.

4. SUBMITTING AN APPLICATION

- 4.1 The rescheduling application form Application Form Rescheduling of a Substance or Medicine (**GLF-CEM-NS-01A**) accessible on the SAHPRA website, must be submitted to SAHPRA as per section 4.2 below.
- 4.2 Variation applications for the rescheduling of a registered medicine or substance (Type II) for evaluation must be submitted via the SAHPRA Engagement Portal (refer to the SAHPRA latest eCTD Specification (**SAHPGL-HPA-09**) for more information as accessible on the SAHPRA website).
- 4.3 The applicant must ensure that all relevant fields are completed on the application form (**GLF-CEM-NS-01A**) and that all supporting documentation is attached. Incomplete applications will be identified as deficient, and the evaluation will not progress until deficiencies are addressed.
- 4.4 The prescribed application fee and proof of payment must accompany the application. For the current fee payable, refer to the latest fee schedule as published in the Government Gazette and published on the SAHPRA website <https://www.sahpra.org.za/government-gazette/>.
- Payments should be made as per the latest SAHPRA Payment Guideline (**SAHPGL-FIN-01**).
- 4.5 In the event of a rescheduling application for a product moving to a lower schedule, a unique proprietary name and a separate registration dossier is required.
- 4.6 For rescheduling an existing registered product to a lower schedule which is part of an existing dossier, a delinking from the master dossier will be required.

- 4.7 The proprietary name change variation application for the de-linked medicine, should be submitted as the next follow-up sequence following an approval of a rescheduling application and the applicant should amend the eCTD envelope to remove the details of the master.

5. TIMELINES FOR PROCESSING OF RESCHEDULING APPLICATIONS

5.1 Rescheduling:

- 5.1.1 For Rescheduling Type II variation applications made to SAHPRA, the timeline will be 220 working days.

5.2 Letter to the applicant:

- 5.2.1 A correspondence letter will be sent to the applicant regarding the outcome of an application, and
- 5.2.2 If an application does not meet the evaluation criteria, a query letter will be sent to the applicant stating the deficiencies identified within the application.

5.3 Permittable query rounds:

- 5.3.1 The query rounds will be as per General Information guideline (**SAHPGL-HPA-07**), consultations with the applicant can be done where applicable.
- 5.3.2 If the applicant fails to address the queries adequately by the within the permissible query rounds, the application will be tabled for rejection.
- The applicant will be required to submit a new application, and the applicable fee shall be payable.
- 5.3.3 Each response timeframe should be within 20 working days of receipt and notification of recommendation variations@sahpra.org.za. Applicants may submit a written request for extension, addressed to variations@sahpra.org.za and namesandscheduling.variations@sahpra.org.za.

5.4 Scheduling of medicines as approved and gazetted by the Minister of Health

- 5.4.1 Rescheduling recommendations that SAHPRA has approved will be submitted as an amendment to the published Schedules, for consideration by the Minister of Health and publication in the *Government Gazette*.

- 5.4.2 Applicants are required to update their PI/ PIL and all relevant sections of the dossier for the registered product to align with the new requirements as published and gazetted by the Minister of health.

6. REFERENCES

The following related documents are referenced:

- 6.1 SAHPGL-CEM-NS-02_Guideline to the Scheduling of Substances and Medicines
- 6.2 GLF-CEM-NS-01A_ Application Form _Rescheduling of a Substance or Medicine
- 6.3 SAHPGL-HPA-09_SAHpra eCTD latest Specification
- 6.4 GLF-CEM-NS-01A _Application Form Rescheduling of a substance or Medicine
- 6.5 SAHPGL-HPA-07_ General Information guideline

7. VALIDITY

This guideline is valid for a period of 5 years from the effective date of revision and replaces the previous guideline on application, rescheduling of substances and medicines (SAHPGL-CEM-NS-01_v1). It will be reviewed on this timeframe or as and when required.